



1. NO-HODGWIG sign

WHAT YOU SEE

'NO-HODGWIG' wig sign

WHAT IT ENCODES

Non-Hodgkin lymphoma is a heterogeneous family of lymphoid malignancies of B-, T-, and NK-cell origin — far more common and more biologically varied than Hodgkin lymphoma. Unlike Hodgkin (Reed-Sternberg cells, contiguous nodal spread, bimodal age peaks), NHL spreads non-contiguously/hematogenously, more often involves extranodal sites, and frequently presents at advanced stage. Mnemonic: 'No Reed-Sternberg, no Hodgkin' — NHL is everything else, mostly B-cell.

BOARD TRAP

Reed-Sternberg cells (CD15+, CD30+, 'owl-eye') indicate HODGKIN, not NHL — putting RS cells in an NHL stem is the trap. NHL also tends to skip nodal regions (non-contiguous spread) and involve extranodal sites, whereas Hodgkin spreads contiguously node-to-node.

2. Census 80,550

WHAT YOU SEE

Lit-up census counter '80,550'

WHAT IT ENCODES

NHL is among the most common cancers (roughly 80,000+ new US cases per year and a top-10 cancer by incidence). The overwhelming majority — about 85-90% — are of B-cell lineage, with T-cell and NK-cell lymphomas making up the minority. Risk rises with age, immunosuppression, autoimmune disease, and certain infections. Mnemonic: the census counts tens of thousands a year, and ~9 in 10 are B-cell.

BOARD TRAP

Assuming most lymphomas are T-cell is the trap — NHL is predominantly B-cell (~85-90%). T-cell lymphomas are the minority and tend to be more aggressive and harder to treat (e.g., peripheral T-cell lymphoma, ATLL, cutaneous T-cell lymphoma).

3. DLBCL 31% (left)

WHAT YOU SEE

Red 'DLBCL 31%' on LEFT census board

WHAT IT ENCODES

Diffuse large B-cell lymphoma is the MOST COMMON aggressive NHL and the most common NHL overall (~30%). It presents as a rapidly enlarging nodal or extranodal mass, often with elevated LDH and B symptoms, and is potentially CURABLE with R-CHOP immunochemotherapy (cures ~60%). Cell-of-origin (GCB vs ABC) and double-hit (MYC + BCL2/BCL6) status refine prognosis. Mnemonic: DLBCL = Big aggressive lymphoma, R-CHOP its cure.

BOARD TRAP

Calling follicular lymphoma the most common aggressive NHL is the trap — FL is the most common INDOLENT NHL, while DLBCL is the most common aggressive one (and most common overall). Rapid growth + high LDH + curable-with-R-CHOP describes DLBCL, not indolent FL.

4. FL 22% (right)

WHAT YOU SEE

Orange 'FL 22%' on RIGHT census board

WHAT IT ENCODES

Follicular lymphoma is the MOST COMMON indolent NHL (~20-22%) and second most common NHL overall. It is driven by t(14;18)/BCL2 overexpression (defective apoptosis), follows a waxing-and-waning, INCURABLE but indolent course, and is managed by watch-and-wait when low-burden or rituximab ± chemo when symptomatic. It transforms to DLBCL at ~3%/year. Mnemonic: FL = the slow grower, t(14;18), incurable but indolent.

BOARD TRAP

Labeling FL aggressive/curable like DLBCL is the trap — FL is indolent and generally incurable; the goal is disease control, not cure. A follicular patient with sudden rapid growth and rising LDH suggests transformation to DLBCL and warrants re-biopsy.

5. Big B 90%

WHAT YOU SEE

Large blue 'B 90%' block

WHAT IT ENCODES

About 85-90% of NHL is of B-cell lineage; the major B-cell entities are DLBCL, follicular, mantle cell, marginal-zone/MALT, Burkitt, and CLL/SLL. T/NK-cell lymphomas (peripheral T-cell lymphoma, ATLL, anaplastic large-cell, cutaneous T-cell/mycosis fungoides, extranodal NK/T nasal) are the aggressive minority. Most B-cell lymphomas express CD20, the target of rituximab. Mnemonic: B for Big majority — and B for the CD20 target of rituximab.

BOARD TRAP

Forgetting that most NHL is CD20+ B-cell (and thus rituximab-responsive) is the trap — T-cell lymphomas do NOT express CD20, so rituximab is ineffective for them, and they generally carry a worse prognosis than their B-cell counterparts.

6. 4% banner

WHAT YOU SEE

'4%' yellow ribbon

WHAT IT ENCODES

NHL incidence has risen over recent decades, driven partly by an aging population and by immunosuppression. Key risk states: HIV/AIDS, post-transplant immunosuppression (PTLD), congenital immunodeficiency, autoimmune disease (Sjögren, Hashimoto, celiac, rheumatoid arthritis), and chronic infections/antigen stimulation. Mnemonic: immune dysregulation + age = rising NHL.

BOARD TRAP

Overlooking immunosuppression/HIV as a major NHL risk factor is the trap — HIV-associated lymphomas (DLBCL, Burkitt, primary CNS lymphoma, primary effusion lymphoma) are AIDS-defining, and autoimmune disease raises risk of specific lymphomas (e.g., Sjögren → MALT/parotid lymphoma).

7. 74% 5-yr survival

WHAT YOU SEE

'SURVIVAL 74% 5-YR' label

WHAT IT ENCODES

Overall 5-year survival for NHL is favorable (~70%+), but this aggregate masks subtype-driven differences in the curability paradox: aggressive lymphomas (DLBCL, Burkitt) are potentially CURED with chemo, while indolent lymphomas (follicular, mantle cell) are typically INCURABLE but slow and relapsing. Mnemonic: good odds overall, but the path differs — aggressive=cure-or-fail, indolent=control-and-relapse.

BOARD TRAP

Reading survival statistics as if all NHL behaves the same is the trap — prognosis and treatment intent are subtype-specific. Telling a patient with indolent FL they will be 'cured,' or telling a DLBCL patient their disease is merely chronic, both misstate the biology.

8. Rogues' Gallery — EBV

WHAT YOU SEE

'EBV' wanted poster

WHAT IT ENCODES

Epstein-Barr virus is linked to multiple lymphomas: endemic Burkitt (~95% EBV+), HIV-associated lymphomas and primary CNS lymphoma, post-transplant lymphoproliferative disorder (PTLD), extranodal NK/T-cell lymphoma (nasal type), and EBV+ DLBCL. EBV immortalizes B cells via latent membrane proteins, especially when T-cell surveillance is impaired. Mnemonic: EBV = Burkitt (endemic), PTLD, CNS lymphoma, NK/T nasal.

BOARD TRAP

Forgetting EBV behind PTLD is the trap — in a transplant recipient with new adenopathy/mass, reducing immunosuppression (± rituximab) is first-line because the EBV-driven proliferation may regress. Also note sporadic (Western) Burkitt is usually EBV-NEGATIVE, unlike endemic Burkitt.

9. HTLV-1

WHAT YOU SEE

'HTLV-1' wanted poster

WHAT IT ENCODES

Human T-lymphotropic virus type 1 causes Adult T-cell leukemia/lymphoma (ATLL), endemic to southwestern Japan, the Caribbean, and parts of Africa. Classic features: CD4+ T-cell malignancy, HYPERCALCEMIA (lytic bone lesions), skin lesions, lymphadenopathy, and circulating 'flower cells' (multilobated nuclei). Mnemonic: HTLV-1 → ATLL → T cells, hypercalcemia, flower cells.

BOARD TRAP

Attributing ATLL hypercalcemia to bony metastasis rather than tumor-driven (PTHrP/cytokine) mechanisms is the trap. Also, HTLV-1 causes a T-CELL malignancy — confusing it with the B-cell lymphomas is an error, and its hypercalcemia + flower cells + endemic geography are the giveaways.

10. HCV

WHAT YOU SEE

'HCV' wanted poster

WHAT IT ENCODES

Hepatitis C virus is associated with marginal-zone lymphomas, especially splenic marginal-zone lymphoma, through chronic B-cell antigen stimulation; it also causes mixed cryoglobulinemia. Eradicating HCV with direct-acting antivirals can induce lymphoma regression, so HCV testing is part of the lymphoma workup. Mnemonic: HCV → splenic/marginal-zone lymphoma — treat the virus, shrink the tumor.

BOARD TRAP

Going straight to chemotherapy for an HCV-positive marginal-zone lymphoma is the trap — antiviral therapy alone can produce remission and is first-line in HCV-driven disease. Missing the HCV link means missing a non-cytotoxic, potentially curative option.

11. H. pylori

WHAT YOU SEE

'H. pylori' stomach poster

WHAT IT ENCODES

Helicobacter pylori drives gastric MALT (extranodal marginal-zone) lymphoma via chronic antigenic stimulation. For early-stage, H. pylori-positive gastric MALT, antibiotic ERADICATION (triple/quadruple therapy) alone induces regression/cure in most patients. Mnemonic: H. pylori → gastric MALT → kill the bug, cure the lymphoma.

BOARD TRAP

Treating t(11;18)-POSITIVE gastric MALT with antibiotics alone is the trap — those tumors do not respond to eradication and need radiation or systemic therapy. Antibiotic cure applies only to early-stage, H. pylori-positive, t(11;18)-negative gastric MALT.

12. HHV-8

WHAT YOU SEE

'HHV-8' wanted poster

WHAT IT ENCODES

Human herpesvirus 8 (KSHV) causes primary effusion lymphoma (a body-cavity lymphoma presenting as malignant pleural/pericardial/peritoneal effusions WITHOUT a mass, usually in HIV/AIDS) and is also linked to Kaposi sarcoma and multicentric Castleman disease. Mnemonic: HHV-8 → primary effusion lymphoma + Kaposi + Castleman.

BOARD TRAP

Working up an HIV patient's lymphomatous effusion as routine malignant effusion and missing primary effusion lymphoma (HHV-8-driven, often EBV co-infected, no nodal mass) is the trap. HHV-8 is the same virus behind Kaposi sarcoma, a useful linking clue.

13. Ann Arbor staging map

WHAT YOU SEE

Body 'Ann Arbor I-IV staging map'

WHAT IT ENCODES

Ann Arbor (Lugano-modified) staging: I = one nodal region; II = ≥2 regions same side of diaphragm; III = both sides of diaphragm; IV = disseminated/extranodal (marrow, liver, lung). The suffix 'B' denotes B symptoms — fever >38°C, drenching NIGHT SWEATS, and unexplained WEIGHT LOSS >10% over 6 months — which worsen prognosis; 'A' means none. PET-CT is standard for staging FDG-avid lymphomas. Mnemonic: I-II-III-IV by diaphragm sides; 'B' for the bad symptoms.

BOARD TRAP

Counting pruritus, fatigue, or alcohol-induced pain as 'B symptoms' is the trap — only fever, drenching night sweats, and >10% weight loss qualify. Staging upstages disease that crosses the diaphragm or involves extranodal/marrow sites, which changes treatment intensity.

14. HBV screen mandatory

WHAT YOU SEE

'HBV SCREEN MANDATORY' checklist

WHAT IT ENCODES

Hepatitis B screening (HBsAg, anti-HBc) is MANDATORY before rituximab or other anti-CD20 therapy: depleting B cells can REACTIVATE latent HBV and cause fulminant hepatitis and death. Patients who are HBsAg-positive or anti-HBc-positive should receive antiviral prophylaxis (e.g., entecavir or tenofovir) during and after therapy. Mnemonic: before rituximab, check Hep B — or risk fulminant reactivation.

BOARD TRAP

Giving rituximab without HBV screening is a high-yield safety trap — anti-HBc-positive patients (even with cleared surface antigen) can reactivate. The correct action is screen first and start antiviral prophylaxis in those at risk, not simply proceed.

15. IPI / FLIPI scores

WHAT YOU SEE

'IPI A-P-L-E-S' and 'FLIPI' risk panels

WHAT IT ENCODES

The International Prognostic Index (IPI) risk-stratifies aggressive lymphoma (DLBCL) using five factors — Age >60, LDH elevated, ECOG performance status ≥ 2 , Stage III/IV, and Extranodal sites >1 ('APLES'). FLIPI is the follicular-specific index (factors: Age, LDH, Hemoglobin, Stage/nodal areas), and MIPI is the mantle-cell index. Mnemonic: IPI 'APLES' for aggressive DLBCL; FLIPI for Follicular; MIPI for Mantle.

BOARD TRAP

Applying the IPI to follicular lymphoma (or vice versa) is the trap — use FLIPI for follicular, MIPI for mantle cell, and IPI for DLBCL. Each index uses different weighted variables, so the wrong tool gives the wrong prognosis.
